

SmartCare Multispeciality Hospital — Insurance & TPA Guide

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1. Purpose & Scope

1.1 Purpose

This comprehensive guide provides patients, corporate policyholders, and family caregivers with explicit guidelines regarding health insurance claims, cashless hospitalization, Third Party Administrator (TPA) coordination, pre-authorization workflows, required claim documentation, and reimbursement procedures at SmartCare Multispeciality Hospital.

1.2 Scope

This policy applies to all planned and emergency inpatient hospitalizations, day-care procedures, surgical treatments, and authorized outpatient diagnostic packages covered under private health insurance, corporate group medicaid policies, Government health schemes, and public sector insurance providers empaneled with SmartCare Hospital.

2. Core Insurance Concepts & Terminology

To navigate insurance coverage smoothly, patients should understand the following key terms:

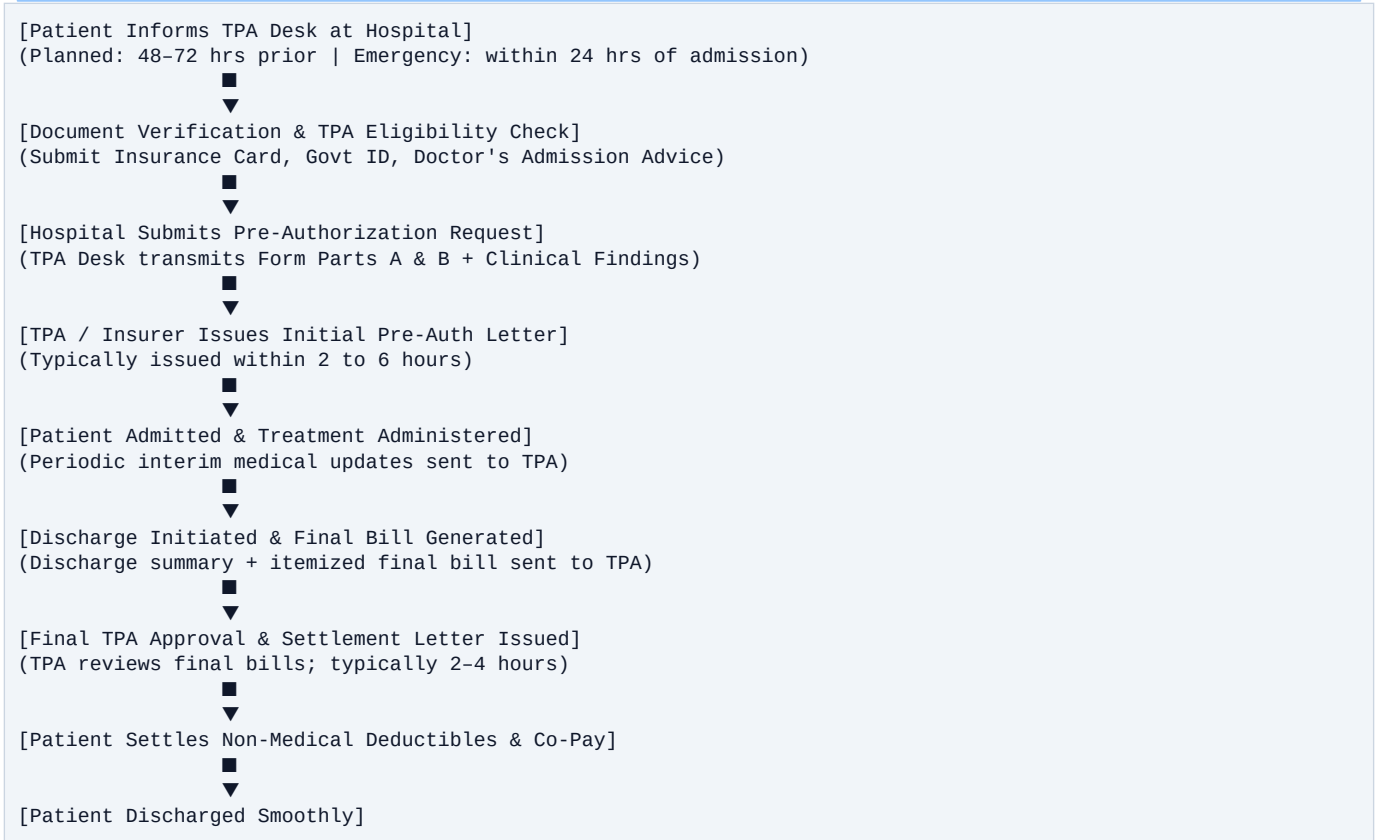
- **Cashless Hospitalization:** A facility wherein the insurance provider or TPA pays the approved hospital bills directly to SmartCare Hospital, subject to policy terms. The patient pays only for non-medical expenses, deductibles, and co-payment portions.
 - **Third Party Administrator (TPA):** An organization licensed by the Insurance Regulatory and Development Authority (IRDAI) that processes health insurance claims and coordinates pre-authorizations on behalf of insurance companies.
 - **Network Hospital:** A healthcare facility empaneled with an insurer or TPA to provide pre-negotiated treatment rates and cashless facilities. SmartCare Hospital is a Preferred Provider Network (PPN) hospital for over 35 leading insurance companies and TPAs.
 - **Pre-Authorization (Pre-Auth):** Formal advance approval granted by the insurer/TPA agreeing to settle treatment expenses up to a designated initial financial limit based on estimated hospitalization costs.
 - **Co-Payment (Co-Pay):** A policy clause requiring the insured patient to pay a designated percentage (e.g., 10%, 15%, or 20%) of the total admissible claim amount.
 - **Deductible:** A fixed upfront amount specified in the policy that the patient must pay before insurance coverage commences.
 - **Non-Admissible / Non-Medical Expenses:** Items not reimbursed by insurance companies according to IRDAI guidelines (e.g., administrative admission fees, registration charges, thermometer, extra food, toiletries, attendant meals, luxury room charges).
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3. Empaneled Insurance Companies & TPAs

SmartCare Multispeciality Hospital is empaneled with all major government and private health insurance entities:

Category	Empaneled Entities (Examples)
Public Sector Insurers	National Insurance, New India Assurance, Oriental Insurance, United India Insurance
Private Standalone Health Insurers	Star Health, Care Health Insurance, HDFC ERGO, Niva Bupa, ManipalCigna, Aditya Birla Health
General Insurance Companies	ICICI Lombard, Bajaj Allianz, Tata AIG, SBI General, Reliance General, IFFCO Tokio
Major Third Party Administrators (TPAs)	Medi Assist TPA, Paramount Health Services, Vidal Health TPA, Heritage Health, MDIndia Health Insurance TPA, Raksha TPA, Family Health Plan (FHPL)
Government & Corporate Schemes	CGHS (Central Govt), ECHS (Ex-Servicemen), Telangana State Government Employees Scheme, Major Corporate Groups

4. End-to-End Cashless Hospitalization Workflow



5. Required Documents for Cashless Claims

- To prevent processing delays, the patient or attendant must submit the following documents at the **TPA & Cashless Helpdesk (Ground Floor, Wing B)**:
- Valid Health Insurance E-Card or Physical Card** (showing Policy Number and Insured Name).
 - Current Policy Copy / Schedule** (first page showing sum insured, inception date, and sub-limits).
 - Government-Issued Photo ID of the Patient** (Aadhaar Card, PAN Card, Passport, or Driving License).
 - Government-Issued Photo ID of the Primary Policyholder** (if patient is a dependent).
 - Treating Doctor's Official Admission Requisition Slip** (stating provisional diagnosis, proposed treatment/surgery, and estimated length of stay).
 - Relevant Diagnostic Investigation Reports** (ultrasound, CT, MRI, biopsy, or blood reports supporting the need for admission).
 - Duly Signed Cashless Pre-Authorization Form** (Part A signed by patient/attendant; Part B completed by hospital treating team).
 - Corporate Employee ID Card** (if claiming under an employer group mediclaim policy).

6. Emergency Cashless Hospitalization Procedure

In acute medical emergencies (e.g., myocardial infarction, severe trauma, stroke, acute appendicitis):

- **Immediate Clinical Care First:** Medical stabilization takes immediate priority over insurance paperwork. Emergency doctors will admit and treat the patient immediately without waiting for pre-authorization approval.
- **Reporting Window:** The patient's attendant must notify the TPA Desk at the Emergency Counter **within 24 hours of emergency admission** (or before discharge, whichever is earlier).
- **Emergency Pre-Auth Submission:** The hospital TPA coordinator will transmit the emergency clinical notes, vital parameters, and pre-authorization form stamped as "EMERGENCY ADMISSION" to the insurer/TPA.
- **Temporary Security Deposit:** A refundable provisional security deposit may be collected at admission until the initial pre-authorization approval letter is received from the insurer. Upon receipt of approval, the deposit is credited back or adjusted against non-medical expenses.

7. Reimbursement Claims Workflow (Non-Network / Cashless Denials)

If a patient's insurance company is not empaneled for cashless facilities, or if cashless approval is declined due to policy ambiguities, the patient can file for **Direct Reimbursement**:

1. **Pay Hospital Bills Directly:** The patient settles the hospital bill upon discharge via card, cash, UPI, or bank transfer.
2. **Collect Original Claim Dossier:** At the time of discharge, the SmartCare Billing & MRD Desk will provide a complete, indexed **Reimbursement Dossier** comprising:
 - Original final itemized hospital bill with receipt stamp.
 - Original payment money receipts.
 - Original signed discharge summary.
 - Detailed medicine purchase bills with batch numbers and doctor's prescriptions.
 - All original diagnostic reports (blood, histology, radiology films and reports).
 - Implant invoice with outer sticker and serial number (for orthopedic/cardiac implants).
 - Duly signed and stamped Insurance Claim Form (Part A & B).
3. **Submission to Insurer:** Submit the complete dossier to your TPA/insurer within **15 to 30 calendar days** from the date of discharge.
4. **Claim Settlement:** The insurer audits the documents and deposits the approved amount directly into the policyholder's bank account via NEFT within 15–30 days.

8. Common Reasons for Pre-Auth Delays & Denials

Understanding common insurance bottlenecks helps patients avoid unnecessary delays:

Reason for Delay / Denial	Contributing Factors	How to Avoid / Resolve
Missing Past Medical History	Insurer requests first consultation paper to check for pre-existing illness (PED).	Provide old treatment files, past prescription slips, and previous discharge summaries.
Ailment Within Waiting Period	Specific diseases (e.g., hernia, cataract, joint replacement) often have 1–2 year waiting periods.	Check policy schedule for specific 24-month waiting clauses prior to planned admission.
Discrepancy in Name / Age	Spelling variation between Aadhaar card and Insurance card.	Provide an official affidavit or correction letter from employer/broker.
Query on Necessity of Admission	Insurer questions whether treatment could have been administered in OPD.	Treating consultant provides medical justification letter proving necessity of indoor stay.
Non-Disclosed Pre-Existing Disease	Undisclosed diabetes, hypertension, or cardiac history.	Always fully disclose chronic medical history when purchasing health insurance.
Delayed Discharge Final Approval	TPA medical auditor takes 2 to 4 hours to review final bills and pharmacy records.	Hospital submits discharge paperwork promptly; attendants are advised to budget 3 hours for discharge.

9. Non-Admissible Items (Patient Out-of-Pocket Expenses)

Under IRDAI regulations, health insurance policies do not cover certain routine overheads and personal hygiene consumables. Common non-covered items include:

- Admission registration fees and medical record documentation charges.
- Personal hygiene kits, sanitary napkins, dental kits, combs, diapers, and baby wipes.
- Thermometers, pulse oximeter probes, crepe bandages, and gloves (subject to specific policy clauses).
- Attendant meals, cafeteria bills, and guest telephone charges.
- Nutritional supplements, vitamins, and protein powders (unless prescribed as an integral therapeutic drug during hospitalization).
- Room rent tariff differences if the patient chooses a room category exceeding their policy eligibility.

10. RAG Knowledge Search & Question Triggers

- **Trigger Queries:** "Does SmartCare accept Star Health / HDFC Ergo / Care Health?", "How do I get cashless hospitalization?", "What documents do I need for TPA?", "Why is my insurance pre-auth taking so long?", "What are non-medical charges?", "Can I claim insurance for day-care surgery?", "How do I file for insurance reimbursement?", "Does insurance cover room rent?"
- **Standardized Fact Base:** SmartCare is empaneled with major public and private TPAs. Cashless requires health card, ID, and doctor's admission advice. Planned admission requests must be submitted 48–72 hours in advance. Emergency pre-auth must be notified within 24 hours. Non-medical expenses must be settled directly by the patient.

11. Frequently Asked Questions (FAQs)

Q1: How do I know if my health insurance company provides cashless facility at SmartCare?

A: You can visit the TPA Helpdesk on the Ground Floor (Wing B), call our insurance desk at +91-40-6999-2233, or check the list of empaneled network insurers on our website.

Q2: How many days in advance should I submit documents for a planned surgery cashless claim?

A: For planned surgeries or admissions, please submit your insurance documents at least **48 to 72 hours prior** to the admission date to obtain pre-authorization approval before you arrive.

Q3: What happens if my insurance pre-authorization is not approved before my planned surgery?

A: You may either postpone the admission until approval is received or pay an admission deposit under the cash category and later convert to cashless upon receiving approval or claim reimbursement after discharge.

Q4: How long does a TPA take to approve an initial pre-authorization request?

A: TPAs typically respond within 2 to 6 hours after receiving complete medical notes, clinical history, and cost estimates from the hospital.

Q5: Is emergency hospitalization covered under cashless insurance?

A: Yes. In emergency situations, medical treatment starts immediately. Your attendant must submit insurance documents at the TPA desk within 24 hours of admission to initiate cashless processing.

Q6: Why do I have to pay a security deposit if I have cashless insurance?

A: An initial refundable deposit is collected to cover non-admissible expenses not paid by insurance (toiletries, registration, non-medical items) or to secure admission while pre-authorization approval is pending.

Q7: What is room rent capping and how does it affect my bill?

A: Many policies limit room rent to 1% or 2% of the sum insured per day. If you choose a room above your eligible limit, proportionate deductions will apply across doctor fees, nursing charges, and surgery costs.

Q8: What are non-medical expenses?

A: Non-medical expenses are items excluded by IRDAI regulations, including administrative charges, gloves, personal hygiene kits, attendant food, and telephone calls. These must be paid directly by the patient.

Q9: Can I claim insurance for day-care procedures (e.g., cataract surgery, dialysis, chemotherapy)?

A: Yes. Many modern policies cover advanced day-care procedures requiring less than 24 hours of hospitalization. Prior pre-authorization is required.

Q10: Why is there a delay during the final insurance clearance on the day of discharge?

A: Once the doctor signs the discharge summary, the hospital compiles the final itemized bill, lab reports, and pharmacy records and sends them to the TPA. The TPA's medical audit team takes approximately 2 to 4 hours to verify and issue the final settlement letter.

Q11: What is a co-payment clause?

A: A co-payment clause is an agreement in your policy where you agree to pay a fixed percentage (e.g., 10% or 20%) of the total admissible hospital bill, while the insurer covers the remaining percentage.

Q12: What should I do if my cashless claim is rejected by the TPA?

A: If cashless is denied, it does not mean your claim is invalid. You will need to settle the hospital bill directly upon discharge, and SmartCare will provide a complete original reimbursement claim docket for you to submit directly to your insurer.

Q13: Does SmartCare accept Central Government Health Scheme (CGHS) or ECHS?

A: Yes, SmartCare is recognized for CGHS, ECHS, and Telangana State Government health schemes for eligible empaneled specialties. Relevant referral letters and beneficiary cards are mandatory.

Q14: Can I use two different health insurance policies for the same hospitalization?

A: Yes. You can use your primary policy for initial cashless clearance. After discharge, SmartCare will provide attested copies of bills and a settlement certificate showing the balance amount, which you can submit to your secondary insurer for reimbursement.

Q15: What documents do I need to bring for insurance verification?

A: Bring the physical insurance card or e-card, policy schedule, patient's government photo ID (Aadhaar/PAN), primary insured's ID, and treating doctor's admission note.

Q16: Are pre-hospitalization and post-hospitalization diagnostic tests covered?

A: Most comprehensive policies cover relevant medical expenses incurred 30 days prior to admission and 60 to 90 days after discharge. These are claimed via post-discharge reimbursement directly with the insurer.

Q17: Does insurance cover maternity and childbirth expenses?

A: Maternity coverage depends on your specific insurance policy terms. Many retail policies exclude maternity or enforce a 2 to 4 year waiting period. Corporate group policies frequently include maternity benefits.

Q18: What is a pre-existing disease (PED) query?

A: Insurance companies often request the first prescription or past medical records to determine whether a condition (such as diabetes, hypertension, or heart disease) existed before the policy was purchased.

Q19: Can I upgrade my room from a Twin Sharing to a Single Private room under insurance?

A: Yes, provided you agree in writing to pay the difference in room rent and the associated proportionate increase in medical and surgical charges not covered by your policy.

Q20: What is an enhancement request in cashless insurance?

A: If the initial pre-authorization amount is exhausted during treatment due to complications, extended ICU stay, or additional surgery, the hospital TPA team sends an "Enhancement Request" with clinical justification to increase the approved limit.

Q21: Does insurance cover robotic surgery?

A: Most insurers cover modern robotic and minimally invasive surgeries under IRDAI modern treatment guidelines, but specific sub-limits or co-pays may apply. Check with the TPA desk beforehand.

Q22: Who decides whether my cashless claim is approved—the hospital or the insurance company?

A: The decision to approve, query, or deny a cashless claim rests entirely with your insurance company or TPA. SmartCare acts strictly as a healthcare provider and liaison.

Q23: How can I track the status of my cashless claim while admitted?

A: You can visit the TPA Helpdesk on the Ground Floor, check the SmartCare Patient Portal, or use your TPA's mobile app using your claim/pre-auth reference number.

Q24: Can an outpatient consultation be covered under cashless insurance?

A: Standard health insurance policies cover inpatient admissions exceeding 24 hours and specific day-care procedures. Outpatient (OPD) doctor consultations are covered only if your policy has a specific OPD rider.

Q25: What happens to my original medical reports if I am claiming reimbursement?

A: Insurance companies require original lab reports, discharge summaries, and bills for reimbursement. SmartCare will provide certified duplicate copies for your personal records before you submit the originals.

12. Official Contact Directory

- **Corporate TPA & Cashless Helpdesk:** Ground Floor, Wing B
- **Direct Phone Lines:** +91-40-6999-2233 / +91-40-6999-2234
- **Email:** tpa@smartcarehospital-demo.org / insurance@smartcarehospital-demo.org
- **Operating Hours:** Monday–Saturday, 8:00 AM – 7:00 PM (Emergency pre-auth handled 24/7 at ER desk)